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Essay 2: Rhetorical Analysis of Cedar Rapids v. Garret F.

In the years since the federal government started funding special education with the Individuals with Disabilities Education Act (IDEA), the amount of students in special education has increased (United States, National Center for Education Statistics, "Children").¹ Not only this, but the amount of students that are 'medically fragile' (i.e. dependent on medical technology) has also increased. This is not new either, it was already happening and noticed in 1998, as better medical technology let more children survive through accidents or conditions they earlier would not have (Bartlett 1). As students required more care, schools were required to provide more care under IDEA. However, some care could be costly and IDEA exempted 'medical services' from what schools were required to provide (United States, Supreme Court 15). This led to tension between the school districts (~~wanting to lower costs~~) and families of disabled students (~~wanting access to a free public education~~) over what constituted 'medical services'. Those in support of the families argued that the disabled children, same as all other children, should be able to have an education. ~~In opposition~~ On the other hand, those in support of the school districts argued the schools were places primarily for education, not care for care with disabilities. The people on the side of the families also argued legal precedent, that Congress's intent and previous court cases implied a smaller definition of medical services. The school districts argued that would be too costly. This conflict was brought to a head and resolved in 1999 in *Cedar Rapids School District v. Garret F.*, a court case between Garret Frey and the school district he attended: ~~*Cedar Rapids School District v. Garret F.*~~ Overall, the federal government should provide ample support to school districts in order to instruct children with disabilities in school and public

¹The citation did not have enough information to uniquely identify one item in the works cited. (#3)

schools should take measures up to but not including hiring a physician/hospital to educate students with disabilities *in classrooms* with other non-disabled students.

This topic/issue, and the popular news coverage of it, is primarily for and oriented towards educators and educative administrators. This is especially true for educators in Iowa ~~New York~~ *the New England Area*. In *Cedar Rapids*, the majority opinion brings up the “legitimate financial concerns” the districts have concerning the controversy (United States, Supreme Court 12). Additionally, in Amicus Curiae brief filed by the National School Boards Association, they make specific mention of the possible cost, describing some costs as “overwhelming” (23). Together these paint a picture of how much school budgets and are affected by this case. This effect would lead to the case being of much more importance to those concerned with school financials, i.e. school administrators, than the average person. ~~Along with the issues of cost~~ *Furthermore*, it can be seen this controversy is oriented towards educators through the popular news coverage. In an article by George Clowes covering the case in *The Heartland*, the language used skews towards the school districts. The article begins mentioning “school district budgets” before describing the case at all, and ends with the school districts estimated cost. This shows a bias towards the school districts, and seems to indicate that this public news coverage was written with an audience of school administrators/educators in mind. *This specific issue is also more important for educators in New England because public schools there have some of the highest rates of students in special education (United States, National Center for Education Statistics, "Students With Disabilities")*

One of the major ways public media argued for greater public school responsibilities was through a focus on the students with disabilities and their condition, providing a more human counter to the drier legal arguments. In an article covering the case, New York Times reporter Linda Greenhouse refers to Frey as “a bright student”, “injured in a childhood accident” who required care to attend school at all. By focusing on Frey’s positive traits and classifying him as a ~~just~~ *just* “a bright student” and not a “disabled student” or even “student with disabilities” Greenhouse ~~provides a greater surface~~ *makes it easier* for the audience to connect/empathize with Frey. She then follows this by describing how he requires ~~the~~ *extensive* care to even attend school

at all, contrasting his bright nature with the possibility of him not ~~he doesn't get~~ even having a chance at an education. This ~~helps as pathos, giving~~ ~~gives~~ the audience a more personal reason to ~~care about~~ ~~be concerned with~~ what care schools provide to students while ~~showing~~ at the same time ~~showing~~ its importance. Another example of this strategy can be found in an article from Education Week which gave background on the case before it had been decided. In ~~the~~ ~~his~~ article, Mark Walsh describes the state of disabled children before IDEA as "often [receiving] no educational services at all." Like Greenhouse, Walsh is drawing the reader's attention to the previous state ~~condition~~ of disabled students. However, unlike Greenhouse, ² he takes a broader stance ~~speaking on a whole group of children and a whole class of services~~ ~~and speaks on all students with disabilities~~. This ~~functions as pathos, getting~~ ~~Walsh gets~~ the audience to consider how ~~positively~~ this act has ~~positively~~ affected students and how ~~needed~~ its enforcement/~~continuation~~ is ~~needed~~. The larger categories also help to connect this argument to more than just one person.

In contrast to the emotional appeals used by the 'pro' side ~~agreeing with the Supreme Court decision~~, one of the arguments used by the school districts and those siding with them (the 'con' side) was based on the purpose and focus of public schools ~~&~~ ~~and~~ general education. In a Los Angeles Times article covering the case, Richard Colvin reports on the executive director of the school boards association ~~remarking~~, ~~who remarked~~ that the decision "takes the focus of schools away from being educators and into being medical service providers".³ This ~~phrasing~~, and creation of a dichotomy between "educating" and "providing medical services" reinforces the ideas that schools are just places for education, and following ~~this~~ ~~the~~ decision in /Cedar Rapids/, ~~providing more intensive care~~, ~~causes~~ ~~would cause~~ them to stray from their primary purpose. In a friend of the court brief filed to the supreme court, the National School Boards Association similarly states ~~that~~ "T[t]o provide a wide range of medical services, other programs – direct educational programs – may be sacrificed" (23).⁴ This ~~additionally separates the primary/stated~~

²Fixing issue of missing commas with introductory element. (#2)

³Change the sentence around the quotation so it fits better (#18)

⁴Slightly change the quotation and preceding introduction so it fits better (#18)

purpose of schools “direct education” from providing care/“medical services” reinforces the distinction between education and providing ‘medical services’, while at the same time emphasizing the way they exist in competition with each other. These arguments both refer to the main function of schools, education, as education ⁵ and by the way they do so they in a way which suggests suggests the platonic ideal of a school is an institution just of learning and education. Alongside this suggestion they juxtapose education/educational programs and medical services. This serves to create and draw to the forefront of thought the separation between medical services and pure education. From there, the context and way in which they {{place the two things side by side}} emphasizes how these two things exist in a state of competition, where to increase one the other must be decreased. From these arguments it is implied that the effect of the ‘pro’ ruling in /Cedar Rapids/ would be to draw schools away from what it is to be a school.

The primary argument those arguing for greater school responsibility used was that based on the text of the law & other legal precedent, ⁶ a smaller definition of medical services (and greater school responsibility) was the correct legal course of action, ⁷ and the one most supported by the will of congress. In her article, Greenhouse, when summarizing the core issue of the court case, references the federal law which would compel the school to pay for more care describing it as “the [IDEA], which guarantees a ‘free appropriate public education’ to students with disabilities.” This description of the law, stating it “guarantees” a “free appropriate” education to students, ⁸ emphasizes and brings up the intent of the law, without getting bogged down in the exact details. When this is compared with the fact that Frey required the services to attend school (referenced earlier in the article), ⁹ it clearly suggests/argues the course of action which aligns with the intent of the law, and by extension Congress, is to have the government, through schools, provide the services. In a friend of the court brief from the National Association of Protection and

⁵The commas here indicate the specification of education as the main function of schools is not necessary to the meaning of the sentence, however I mean to highlight the reference to specifically education, so the commas don’t work well (#7 ish)

⁶The comma here is unnecessary (#7)

⁷The comma here is unnecessary/confusing (#7)

⁸The clause here is non-restrictive, and needs to be set off with commas (#11)

⁹This is an introductory element (specifies when the main action occurs), so it needs a comma (#2)

Advocacy Systems and many other advocacy organizations, they reference the external context of disability law, stating the following: “To deprive a small group of students of the opportunity to benefit from attending school runs counter to all of the principles of the IDEA, the ADA, and other disability laws” (29-30). Similar to the argument made in the New York Times article, here the authors make an appeal to the authority of Congress, through the laws they have passed. This appeal is reinforced by the broader base of support it comes from. By using multiple different pieces of legislation, as opposed to just IDEA, the authors show that it is likely that the overall position of Congress on this and similar matters is to provide the needed support to those with disabilities, and the IDEA is not a single outlier but part of a broader pattern.

The primary argument school districts and those sided with them made was that the narrower definition of medical services would be much too costly for schools. For example, the Los Angeles Times reports that the director of the school board association worries the schools will endure “great strain” from the increased costs and responsibilities (Colvin). This phrasing emphasizes the hardship school districts could go through and, when combined with the context, given shortly after, that the federal government “grossly” under funds special education, supports the idea that this decision is unfair.

In a friend of the court brief filed to the supreme court, the National School Boards association brings up the fact that most school districts are quite small and lack the “specialized staff and resources” needed for “complex constant health services.” This works to promote the school districts argument both through an emotional channel and a more logical one. By contrasting the idea of smaller less funded school districts with resources and services described as “specialized” or “complex” and “constant”, they emphasize the possible ridiculous nature of costs small school districts, who likely can’t pay them, would have to pay.

In the article from the Heartland, Clowes writes on the cost estimates: “the total cost [for Frey’s required services] could be over \$50,000 a year.” Here, Clowes gets specific and zooms in on the specific direct consequences of the case, whether or not the school would have to pay for the services Frey needs. This smaller lens (looking at just one student) is then contrasted with the

large sum of money presented. This serves to highlight and magnify the amount of money schools will have to pay to provide necessary services to *all* of their students who need them.

Before this issue (determining what exactly schools were exempt from providing) was raised to the Supreme Court in *Cedar Rapids*, there had already been multiple attempts to address it, even a prior supreme court case, *Irving Independent School District v. Tatro*. While these previous court cases addressed the question of what is medical and what schools must pay for, none succeeded in answering it in a conclusive manner. This left the door open for more argument and litigation. In an article for the Journal of Special Education, Larry Bartlett remarks that the Court in *Tatro* clearly articulated a test which “provided a clear distinction” between “excluded medical services” and services schools were required to provide, but in “an effort to clarify its position” introduced ideas which lead to different decisions in later courts (2). Although this earlier Supreme Court case did attempt to provide a clear distinction between medical and non-medical services, it failed as a complete solution to this issue because it also mentioned and discussed the cost and burden schools would bear if required to provide all services from a physician. This ‘failure’ was also two-fold, as it not only created space for future courts to introduce new cost-affected distinctions between medical and non-medical services, but it also provides support for courts arguing the other ‘side’, that of a clear physician-dependent distinction between medical and non-medical services. Further non-supreme court cases attempted to address this issue, with some, as acknowledged in the majority opinion of *Cedar Rapids*, using a “multifactor test” which considers “the nature and extent of the services” (United States, Supreme Court 7). These courts were basing their decision of the Supreme Court’s discussion of cost and complexity in *Tatro*, which they interpreted as the Supreme Court “[approving] an approach” balancing those two factors (Bartlett 3). Alongside and after these cases which declined to use the test established in *Tatro*, there were courts which departed from previous rulings using a “multifactor burden test” not only because of the precedent in *Tatro* but also because the “uncertainty of [the] result” of a “multifactor burden test” would “encourage future litigation” and be “disruptive to education” (Bartlett 6-7). These cases following *Tatro*,

while attempting to resolve the issue, resulted in further muddying the waters. Although each individually did resolve a specific argument, the history they collectively established increases the difficulty of arguing based on legal precedent, as both sides were given cases to point to as precedent and reasons as to why the others cases were invalid or not to be used (the discussion of cost and complexity in *Tatro* for those opposed to a physician-based definition of medical services, and the arguments about future litigation from smaller court cases for those opposed to a multifactor burden test, alongside the main arguments and decision from *Tatro*). From all these previous attempts, the Court in *Cedar Rapids* did provide a narrow solution to the immediate issue by clearly establishing the 'bright line' physician test as precedent. However, this only solved the legal issue, and left some of the major underlying issue (the funding issue for special education & school's lack of desire to provide costly services) to be dealt with. One of the possible solutions was increased federal funding for IDEA. In a Congressional Research Service report following the court case it is mentioned increased IDEA funding became a congressional issue (4). Additionally, the Senate created amendments to the Education Flexibility Partnership Act of 1999 with the purpose of increasing IDEA funding (United States, Congress 19).

Overall, rhetorically, the ~~"pro" side~~ (the side arguing for a smaller definition of medical services wins) (the 'pro' side). While the arguments about cost and the nature/purpose of schools are good points, the pro sides arguments outweigh them. Their legal argument is clear and works well for the context of the case, pulling both on the authority of previous courts and of congress. Additionally, the ~~pathos~~emotional argument shows the actual people affected by the ruling and the actual harms of a greater definition of medical services. It also serves to counter the argument that schools are just for learning,¹⁰ because it uses the importance of schools as a place to learn as one of its foundations. My personal stance is one that aligns with the 'pro' side. While cost arguments are true, they can be solved with other legislation (and also were somewhat addressed with more legislation after the case), while ~~whereas~~ both the legal arguments & ~~and~~ ~~pathos~~emotional arguments require the pro side to be taken to be resolved. I have a slight personal stake in this

¹⁰Remove unnecessary comma (#7) (it is a subordinating conjunction, it does not get a comma before it)

controversy/court case,¹⁰ as I have personally benefited from special education programs in public school. While my circumstances are nowhere as extreme as Frey's, I do still believe my experiences have given me a greater interest an stake in special education or disability support.

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